

1. Administrative & Study Identification

Full Study Title: A Study of Pembrolizumab (MK-3475) in Combination With Belzutifan (MK-6482) and Lenvatinib (MK-7902), or Pembrolizumab/Quavonlimab (MK-1308A) in Combination With Lenvatinib, vs Pembrolizumab and Lenvatinib, for Treatment of Advanced Clear Cell Renal Cell Carcinoma (MK-6482-012)-China Extension Study **Short Title/Acronym:** MK-6482-012 (China Extension Study) **Protocol Number:** MK-6482-012 **NCT Number:** NCT05899049 **Trial Status:** Active, Not Recruiting **Sponsor Name:** Merck / Merck Sharp & Dohme LLC **Investigational Product:** Pembrolizumab (MK-3475), Belzutifan (MK-6482), Lenvatinib (MK-7902), Pembrolizumab/Quavonlimab (MK-1308A) **Phase of Development:** Phase III **Medical Monitor:** Merck Sharp & Dohme LLC Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and safety of pembrolizumab plus belzutifan plus lenvatinib or pembrolizumab/quavonlimab plus lenvatinib versus pembrolizumab plus lenvatinib as first-line treatment in Chinese participants with advanced clear cell renal cell carcinoma (ccRCC). **Secondary Objectives:** To evaluate Objective Response Rate (ORR), Duration of Response (DOR), and the overall safety and tolerability profiles of the treatment regimens, specifically tracking the number of participants who experience at least one adverse event (AE).

2.2 Background & Rationale Clear cell renal cell carcinoma (ccRCC) is highly driven by the VHL/HIF-2 α pathway and angiogenesis. The standard of care in the first-line setting includes immune checkpoint inhibitors combined with vascular endothelial growth factor (VEGF) targeted therapies. This study aims to evaluate whether the addition of a HIF-2 α inhibitor (belzutifan) or a CTLA-4 inhibitor (quavonlimab) to the established combination of pembrolizumab (PD-1 inhibitor) and lenvatinib (VEGF TKI) provides superior clinical outcomes (Overall Survival and Progression-Free Survival) for Chinese patients with previously untreated advanced ccRCC. The primary hypotheses are (1) pembrolizumab plus belzutifan plus lenvatinib is superior to pembrolizumab plus lenvatinib with respect to progression-free survival (PFS) and overall survival (OS); and (2) pembrolizumab/quavonlimab plus lenvatinib is superior to pembrolizumab plus lenvatinib with respect to PFS and OS.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional Control Type: Active Comparator (Pembrolizumab + Lenvatinib) **Blinding:** Open-label / Randomized **Randomization:** Randomized into three parallel treatment arms

3.2 Planned Enrollment & Duration Target **\$N\$:** Approximately 249 Chinese participants. The China extension study will include participants previously enrolled in China in the global study for MK-6482-012 (NCT04736706) plus those enrolled during the China extension enrollment period. **Number of Sites:** Multicenter (Sites across China) **Estimated Study Start:** 2023 **Estimated Primary Completion:** TBD (Event-driven based on PFS and OS endpoints)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Has histologically confirmed diagnosis of RCC with a clear cell component. 2. Has received no prior systemic therapy for advanced ccRCC. 3. Male participants are abstinent from heterosexual intercourse or agree to use contraception during and for at least 7 days after the last dose of study intervention with belzutifan and lenvatinib. 4. Female participants are not pregnant or breastfeeding and are either not a woman of child-bearing potential (WOCBP) or use a contraceptive method that is highly effective or are abstinent from heterosexual intercourse during the intervention period and for at least 120 days after pembrolizumab or pembrolizumab/quavonlimab or for at least 30 days after the last dose of lenvatinib or belzutifan, whichever occurs last. 5. Has adequately controlled blood pressure with or without antihypertensive medications. 6. Has adequate organ function. 7. Participants receiving bone resorptive therapy must have therapy initiated at least 2 weeks prior to randomization/allocation.

4.2 Exclusion Criteria 1. Has received prior systemic therapy for advanced clear cell renal cell carcinoma. 2. Has a known additional malignancy that is progressing or has required active treatment within the past 3 years. 3. Has had major surgery, other than nephrectomy within 4 weeks prior to randomization. 4. Has known central nervous system (CNS) metastases and/or carcinomatous meningitis. 5. Has received prior radiotherapy within 2 weeks prior to first dose of study intervention. 6. Has hypoxia or requires intermittent supplemental oxygen or requires chronic supplemental oxygen. 7. Has clinically significant cardiac disease within 12 months from first dose of study intervention. 8. Has a history of interstitial lung disease. 9. Has symptomatic pleural effusion; a participant who is clinically stable following treatment of this condition is eligible. 10. Has preexisting gastrointestinal or non-gastrointestinal fistula. 11. Has a diagnosis of immunodeficiency or is receiving chronic systemic steroid therapy or any other form of immunosuppressive

therapy within 7 days prior to the first dose of study treatment. 12. Has a known psychiatric or substance abuse disorder that would interfere with requirements of the study. 13. Has received a live or live-attenuated vaccine within 30 days before the first dose of study drug; killed vaccines are allowed. 14. Has an active autoimmune disease that has required systemic treatment in the past 2 years. 15. Has a history of noninfectious pneumonitis that required steroids or has current pneumonitis. 16. Has an active infection requiring systemic therapy. 17. Has a known history of human immunodeficiency virus (HIV) infection. 18. Has a known history of Hepatitis B. 19. Has radiographic evidence of intratumoral cavitation, encasement or invasion of a major blood vessel. 20. Has clinically significant history of bleeding within 3 months prior to randomization. 21. Has had an allogenic tissue/solid organ transplant.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * Arm A:

Pembrolizumab 400 mg IV once every 6 weeks (Q6W) + Belzutifan 120 mg orally once daily (QD) + Lenvatinib 20 mg orally QD. * Arm B: Pembrolizumab/Quavonlimab (MK-1308A) IV + Lenvatinib 20 mg orally QD. * Arm C (Comparator): Pembrolizumab 400 mg IV Q6W + Lenvatinib 20 mg orally QD.

Route of Administration: Intravenous (IV) and Oral. **Duration of Treatment:** Pembrolizumab administered for up to 18 administrations (approximately 2 years). Belzutifan and lenvatinib administered continuously until progressive disease (PD) or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications allowed for co-morbidities.

Prohibited: Other concurrent investigational agents, live vaccines within 30 days of the first dose, and concurrent systemic immunosuppressive therapy.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor Imaging, Baseline Labs Baseline Day 0 Randomization, First Dose Administration Interim Q6W Treatment Administration, Safety Labs, Tumor Imaging Assessments (per RECIST 1.1) End of Study At PD or Discontinuation Final Safety Follow-up, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Progression-Free Survival (PFS): According to Response Evaluation Criteria in Solid Tumors Version 1.1 (RECIST 1.1) as Assessed by Blinded Independent Central Review (BICR). Defined as the time from randomization to the first documented progressive disease (PD) or death due to any cause, whichever occurs first. Per RECIST 1.1, PD is defined as $\geq 20\%$ increase in the sum of diameters of target lesions, alongside an absolute increase of ≥ 5 mm. The appearance of one or more new lesions is also considered PD. **Overall Survival (OS):** Defined as the time from randomization to death due to any cause.

7.2 Secondary & Exploratory Endpoints * Objective Response Rate (ORR) Per RECIST 1.1 as Assessed by BICR * Duration of Response (DOR) Per RECIST 1.1 as Assessed by BICR * Number of Participants Who Experienced At least One Adverse Event (AE)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard clinical monitoring via laboratory assessments, vital signs, and patient reporting at each visit. Specific monitoring for on-target effects like hypoxia and anemia (associated with belzutifan). **Serious Adverse Events (SAEs):** Reported by the investigator within 24 hours of site awareness. **Data Monitoring Committee (DMC):** External independent committee reviewing unblinded safety and efficacy data.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy endpoints (PFS, OS). Safety Set for all participants receiving at least one dose of the study medication. **Sample Size Justification:** Powered to detect a statistically significant superiority in PFS and OS for the experimental combinations (Arm A and Arm B) over the active comparator (Arm C). **Handling of Missing Data:** Standard oncology censoring rules applied for time-to-event endpoints (PFS and OS) in the event of missing data or loss to follow-up.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring visits to ensure protocol adherence and data integrity. **Audit Trail:** Validated electronic data capture (eCRF) system with full audit trails and active data clarification form management.

11. Study Identifiers & Governance

Primary ID: MK-6482-012 **Registry Number:** NCT05899049 **Sponsor/Lead Organization:** Merck / Merck Sharp & Dohme LLC **Study Title:** A Study of Pembrolizumab (MK-3475) in Combination With Belzutifan (MK-6482) and Lenvatinib (MK-7902), or Pembrolizumab/Quavonlimab (MK-1308A) in Combination With Lenvatinib, vs Pembrolizumab and Lenvatinib, for Treatment of Advanced Clear Cell Renal Cell Carcinoma (MK-6482-012)-China Extension Study **Official Regulatory Title:** An Open-label, Randomized Phase 3 Study to Evaluate Efficacy and Safety of Pembrolizumab (MK-3475) in Combination With Belzutifan (MK-6482) and Lenvatinib (MK-7902), or MK-1308A in Combination With Lenvatinib, Versus Pembrolizumab and Lenvatinib, as First-Line Treatment in Participants With Advanced Clear Cell Renal Cell Carcinoma (ccRCC)

12. Clinical Framework (Renal Cell Carcinoma Specific)

Primary Condition: Advanced Clear Cell Renal Cell Carcinoma (ccRCC) / Carcinoma, Renal Cell **Diagnosis Threshold:** Histologically confirmed diagnosis of RCC with clear cell component **Medical Methodology:** Systemic First-Line Therapy (Immunotherapy + VEGF targeted therapy + novel HIF-2 α /CTLA-4 targeting) **Optimization Target:** Prolonged Progression-Free Survival (PFS) and Overall Survival (OS)

13. Study Procedures & Methodology

Management Pathway: Standard oncology clinical pathway for advanced ccRCC **Education Protocol (HCP):** Protocol training, RECIST 1.1 evaluation criteria, and adverse event management guidelines (e.g., hypoxia management) **Education Protocol (Patient):** Informed consent process, treatment compliance education, and side-effect reporting instructions **Quality Audit Mechanism:** Continuous clinical site monitoring, centralized imaging review (BICR), and data verification

14. Observation & Follow-Up Schedule

Total Duration: Continued until disease progression, unacceptable toxicity, or withdrawal **Onsite Visit Cadence:** Every 6 weeks for IV infusion and clinical assessments **Remote Monitoring Frequency:** Intermittent remote tracking as required between clinical visits **Checklist Review Interval:** At the end of every 6-week treatment cycle

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					